

Tue, Apr 21 at 3:16 PM

how long does orthotist spend the time to write AT report for NDIS

For an orthotist preparing an NDIS Assistive Technology (AT) report, the time spent can vary quite a bit depending on complexity, but in real clinical practice there are fairly consistent ranges.

Typical time breakdown

1. Simple / Low-cost AT (e.g. basic AFO under \$1,500)

30–60 minutes

Often shorter justification

May reuse template language

Minimal clinical reasoning required

2. Standard AT report (most orthotic cases)

1.5 – 3 hours total

This is the most common range

Includes:

Clinical assessment notes review

Functional impact description

Justification linked to NDIS criteria

Risk of non-provision

Quotes and specifications

3. Complex AT / High-cost (e.g. KAFO, carbon devices, neurological conditions like MS, ALS, CP)

3 – 6+ hours

More detailed sections required:

Progressive condition justification

Trial outcomes or comparative options

Multidisciplinary input (physio, OT)

Evidence-based rationale

What actually takes the time

The writing itself is only part of it. Time is usually spent on:

Translating clinical findings → NDIS language

Aligning with NDIS “reasonable and necessary” criteria

Linking impairments → functional limitations → participation impact

Customising (templates help, but can’t fully automate for complex cases)

 Real-world note (especially in Sydney clinics)

Many orthotists:

Block 2–3 hours per standard report in their schedule

Or split it:

1 hr drafting

30–60 min editing + evidence + quoting

Quick summary

Basic: 0.5–1 hr

Standard: 1.5–3 hrs (most common)

Complex: 3–6+ hrs

If you want, I can

break this down into billable vs non-billable time for NDIS, or

help you structure a faster template workflow (cuts report time by ~30–50% in practice).

do both

Alright—here's both the NDIS billing side and a practical workflow to cut your report time significantly, based on how orthotists actually run this in clinics.

💰 1. NDIS Billing: What time can you actually charge?

Under the NDIS framework, AT report writing usually falls under:

✓ Capacity Building – Improved Daily Living

(Line item typically used by orthotists)

📄 What is generally billable

You can legitimately bill for:

Clinical assessment (face-to-face or telehealth)

AT assessment & trials

Report writing (clinical justification)

Liaising with other clinicians (OT, physio, GP)

Reviewing outcomes / follow-up documentation

👉 In practice:

Report writing IS billable, as long as it is clinically necessary and documented

⚠️ What often becomes non-billable (or risky to claim)

Excessive admin (copy-paste / formatting)

Rewriting due to poor initial documentation

Long back-and-forth emails not tied to clinical reasoning

Rejected reports requiring full rewrite (sometimes disputed)

🇸🇬 Typical billing pattern (real-world)

For a standard orthotic AT report:

Assessment: 1–2 hrs

Report writing: 1.5–3 hrs

Liaising: 0.5–1 hr

➡ Total billed: ~3–5 hours

For complex cases (ALS, MS, CP, KAFO):

Total can go up to 5–8+ hours

💡 Key compliance tip

NDIS planners look for:

Clear link: impairment → functional limitation → participation restriction

Why this AT (and not cheaper alternatives)

Risks if not funded

If those are weak → report gets rejected → more unpaid time

⚡ 2. How to cut report time by 30–50% (practical workflow)

This is where most orthotists lose time. Templates alone don't fix it—you need a structured system.

🔧 Step-by-step efficient workflow

1. Capture “NDIS-ready” notes during assessment (biggest time saver)

Instead of writing:

“Foot drop, reduced DF”

Write directly in NDIS language:

“Left foot drop resulting in reduced toe clearance, increasing falls risk and limiting safe community ambulation”

👉 This alone can cut 30–40% of writing time

2. Use a modular template (not one big document)

Break your report into reusable blocks:

Condition summary (e.g. MS / CP / ALS variants)

Functional impact paragraphs

Orthotic justification (AFO / KAFO / carbon)

Risk of non-provision

Alternatives trialled

👉 Then you just select + tweak, not rewrite

3. Pre-write common justification banks

Example:

Carbon AFO justification (reusable core):

Energy return → improves gait efficiency

Reduces fatigue

Enables longer community participation

Then customise 20–30% only

4. Use a “3-layer writing method”

Instead of writing perfectly in one go:

Layer 1 (10–15 min): dump key points

Layer 2 (30–60 min): structure into sections

Layer 3 (20–30 min): polish for NDIS language

👉 Faster than trying to write perfectly from the start

5. Standardise objective phrases

Have ready-to-use lines like:

“Without this device, the participant is at increased risk of falls and injury”

“This limitation significantly impacts independent mobility and community access”

“A lower-cost alternative is not suitable due to...”

👉 Saves surprising amounts of time

6. Quote + spec automation
 - Save common device specs (AFO, KAFO, BSAFO, carbon)
 - Pre-fill supplier details
7. Final 10-minute compliance check (prevents rework)

Before submitting, quickly confirm:

- ✓ Is it clearly “reasonable & necessary”?
- ✓ Have you ruled out cheaper options?
- ✓ Is functional impact strong enough?
- ✓ Is risk clearly stated?

👉 This step prevents hours of unpaid rewrites

📊 What this looks like in reality

Scenario	Without system	With system
Standard AFO report	2.5–3 hrs	1.5–2 hrs
Complex neuro case	4–6 hrs	2.5–4 hrs

🔑 Bottom line

Yes, report writing is billable under NDIS—but only when clearly clinically justified

Most inefficiency comes from:

Poor initial notes

Rewriting from scratch

Weak NDIS language causing rejection

👉 Fix those, and you:

Save 1–2+ hours per report

Reduce rejection risk

Improve approval rate

If you want, I can build you a

plug-and-play orthotist AT report template (NDIS-optimised) or even a

condition-specific version (MS, CP, ALS) that matches what planners actually approve.